

QA Preparation

Huang Guan - bilingual presentation material

Purpose / 用途	Short answers for Friday presentation questions. Use evidence first and keep claims within the MVP boundary.
My QA scope / 我的范围	Slides 1, 8, 9, 20 and 21, plus the final deck check. Darli and Keith present; this document supports my answers.
Current deck note / 当前 PPT	The latest checked deck had 22 slides. Slide 22 is Thank You; the last QA allocation was based on the earlier ranges, so confirm final ownership before presenting.
Safe wording / 安全表述	Educational prototype, synthetic data only, illustrative thresholds, not a diagnosis, not a production privacy or clinical system.
Prepared / 时间	13 August 2026. Sources include the current Google Slides deck, the Render dashboard, GitHub main and the LeanKit board checked in the in-app browser.

How to use this sheet

- Answer the English line first. Add the Chinese note only to remind yourself of the meaning.
- If a question asks for a clinical or production claim, use the Limit line and say it is still pending.
- Do not invent a number, dataset, test result or teammate decision that is not listed here.

1. One-minute system summary

Use this when the examiner asks how the system works.

Frontend	Static HTML, CSS and JavaScript. The live site calls the Render backend; local development uses localhost:5001.
Input	Manual health values. The user reviews them and must tick an explicit confirmation box.
API	POST /api/assess. The backend validates the full request and returns indicators, context, references and an action card.
Integrity	The backend recalculates BMI from height and weight. It does not trust a client-supplied BMI.
Database	DATABASE_URL selects PostgreSQL on Render; SQLite is the local fallback. Confirmed report fields and biomarkers are stored; action_card is not stored.
AI boundary	If OPENROUTER_API_KEY is configured, the action-card layer can call OpenRouter. If unavailable, the rule-based fallback keeps the demo usable.
Live check	Render currently shows both frontend and backend deployed from main@919980d, with healthfirst-team04-db marked Available. GitHub shows no open pull requests.

Q: Why does the user have to confirm the values?

为什么用户必须确认数据？

Suggested answer (EN): The confirmation step makes the user check the final values before they are sent to the backend. It reduces accidental submissions and makes the data flow visible.

中文理解: 确认步骤让用户在发送前检查最终数值, 减少误提交, 也让数据流向更透明。

Evidence: frontend/index.html and app.js; deployed acceptance test passed the confirmation gate.

Limit: This is a user-control step, not medical consent or a full privacy consent process.

Q: What happens after confirmation?

确认后发生什么？

Suggested answer (EN): The frontend sends the confirmed payload to POST /api/assess. The server validates it, recalculates BMI, loads demo benchmarks, builds the indicators and action card, persists the confirmed report, then returns the result.

中文理解: 前端把已确认的数据发送到 API。后端重新验证、计算 BMI、读取演示基准、生成指标和行动卡, 保存确认后的报告, 再返回结果。

Evidence: docs/API.md; backend/app/main.py, assessment.py and persistence.py.

Limit: The current rules and benchmarks are illustrative, not approved clinical rules.

2. Questions for my assigned slides

Slides 1 and 8: introduction, result page and user value.

Q: What problem is HealthFirst solving?

HealthFirst 要解决什么问题？

Suggested answer (EN): HealthFirst turns confirmed screening values into a clear, source-labelled summary and practical next steps. It helps a person understand what may be worth discussing, without presenting a diagnosis.

中文理解: HealthFirst 把用户确认的筛查数值转换成有来源的清晰总结和下一步建议，帮助用户理解哪些内容值得讨论，但不提供诊断。

Evidence: Onboarding deck problem statement and current result-page wording.

Limit: Do not say that the app predicts disease or replaces a doctor.

Q: What does the results page show?

结果页展示什么？

Suggested answer (EN): It shows four indicators - blood pressure, blood glucose, total cholesterol and BMI - with a value, a text status, a shape, a reference and an explanation. Malaysian population context is shown separately from the user's result.

中文理解: 结果页展示四项指标：血压、血糖、总胆固醇和 BMI。每项有数值、文字状态、形状标记、参考来源和解释。马来西亚人口背景数据与个人结果分开显示。

Evidence: API contract and deployed test: four indicators, status legend, Malaysian context and source labels were visible.

Limit: A status is a demo interpretation, not a clinical diagnosis.

Q: What is the benefit compared with asking a general LLM?

与直接使用普通大语言模型相比有什么优势？

Suggested answer (EN): HealthFirst uses a fixed input structure, server-side validation, server BMI recomputation, explicit confirmation, Malaysian context, visible sources and a bounded action-card shape. A general LLM would not automatically provide these project-specific checks or traceability.

中文理解: HealthFirst 有固定输入格式、后端验证、后端 BMI 重算、用户确认、马来西亚背景数据、可见来源和受控的行动卡结构。普通大模型不会自动提供这些项目级检查和可追溯性。

Evidence: docs/API.md, validation.py, action_card.py and the result-page copy.

Limit: This is a workflow and traceability benefit, not proof of clinical accuracy.

Q: Does the AI give the same recommendation every time?

AI 每次都会给出相同建议吗？

Suggested answer (EN): The current PPT only says that an action plan is generated by AI and that the output has a bounded structure. It does not promise fixed or random wording. I would say that the format is controlled, while I would not claim identical text without a live configuration test.

中文理解: 当前 PPT 只说明行动卡由 AI 生成，并且输出有固定结构。它没有承诺文字每次都相同或一定随机。没有实时配置测试时，我不会声称每次文字完全一样。

Evidence: Current deck slide 11: action plan generated by AI and not stored; slide 20: external AI is a separate boundary.

Limit: Do not describe the AI output as a validated clinical method.

3. LeanKit, security and references

Slides 9, 20 and 21: how the team organised work and how we describe limits.

Q: How did LeanKit help the project?

LeanKit 对项目有什么帮助?

Suggested answer (EN): LeanKit made user stories, acceptance criteria, owners and next actions visible. The current board shows Not Started, Started and Finished lanes, plus Huang Guan's SMART Reflection Card and PM cards such as API review, safety review, Innovation and Security Plan.

中文理解: LeanKit 让用户故事、验收标准、负责人和下一步行动可见。当前看到了 Not Started、Started、Finished 三类泳道, 以及 Huang Guan 的 SMART Reflection Card 和 API review、safety review、Innovation、Security Plan 等 PM 卡片。

Evidence: In-app browser check of board 2494590220 and its My Cards view on 13 August 2026.

Limit: The board view did not expose evidence that every acceptance criterion is complete. Do not call all cards Done.

Q: What security controls are actually implemented?

目前真正实现了哪些安全控制?

Suggested answer (EN): The PPT explicitly presents the safety disclaimer and non-diagnostic wording as the implemented boundary. It labels encryption, anonymisation, access control, PDPA review and external-AI governance as planned or conditional, rather than claiming them as complete.

中文理解: PPT 明确把安全免责声明和非诊断措辞列为已实现边界; 加密、匿名化、访问控制、PDPA 审查和外部 AI 治理被列为计划中或有条件的内容, 而不是已完成的功能。

Evidence: PPT slide 20: Security Plan & Risks - MVP Boundary.

Limit: Do not add claims about authentication, encryption or PDPA compliance that are not shown in the deck.

Q: What security controls are not implemented?

哪些安全控制还没有实现?

Suggested answer (EN): The slide says data encryption, data anonymisation, access control and PDPA review are not implemented or not established for this MVP. It also says external-AI data flow needs disclosure if enabled. These are release limits, not completed features.

中文理解: 幻灯片说明数据加密、数据匿名化、访问控制和 PDPA 审查在本 MVP 中尚未实现或尚未建立; 如果启用外部 AI, 还需要披露数据流向。这些是发布限制, 不是已完成的功能。

Evidence: PPT slide 20: the six-item security boundary.

Limit: Never answer that the prototype is production-ready or PDPA-compliant.

Q: What do the references prove?

参考来源能证明什么?

Suggested answer (EN): The references identify DOSM Statistics on Causes of Death, Malaysia 2025, OpenDOSM Deaths by District & Sex, and the four Kaggle clinical datasets named in the deck. They show traceability; they do not approve the project's illustrative thresholds.

中文理解: 参考文献列出 DOSM Statistics on Causes of Death, Malaysia 2025、OpenDOSM Deaths by District & Sex, 以及幻灯片中列出的四个 Kaggle 临床数据集。它们说明来源可追溯, 但不代表批准了项目的演示阈值。

Evidence: PPT slide 21 and the URLs printed on that slide.

Limit: The deck does not show final clinical approval of the thresholds.

4. Database, AI and deployment questions

Use these if the examiner asks for implementation evidence.

Q: Is the deployed version using PostgreSQL?

部署版本是否使用 PostgreSQL?

Suggested answer (EN): The Render dashboard currently shows healthfirst-team04-api and healthfirst-team04-frontend deployed from main commit 919980d, and the PostgreSQL service healthfirst-team04-db marked Available. The PPT describes the intended PostgreSQL tables, but I did not inspect live row counts in this browser check.

中文理解: Render 面板当前显示 healthfirst-team04-api 和 healthfirst-team04-frontend 都从 main 提交 919980d 部署, PostgreSQL 服务 healthfirst-team04-db 状态为 Available。PPT 描述了计划中的 PostgreSQL 表, 但我这次没有检查线上行数。

Evidence: Render dashboard: backend and frontend live on main@919980d; database service Available. PPT slide 11: database architecture.

Limit: The credential is a secret and should never be shown in slides, chat or the PDF.

Q: What data is stored?

数据库保存什么数据?

Suggested answer (EN): The current PPT shows User_Report, Biomarker, Risk_Assessment and Benchmark tables. It also states that the AI-generated action plan is not stored. Use this as the design answer; do not claim live row contents unless they are separately checked.

中文理解: 当前 PPT 展示 User_Report、Biomarker、Risk_Assessment 和 Benchmark 四张表, 并说明 AI 生成的行动计划不保存。可把它作为设计回答, 但没有单独检查时不要声称线上行内容。

Evidence: PPT slide 11: ERD, fields, relationships and 'Action plan generated by AI (not stored)'.

Limit: This is an MVP data design, not an approved retention policy.

Q: Can the action card send health values to a third party?

行动卡会不会把健康数值发送给第三方?

Suggested answer (EN): PPT slide 20 says that if external AI is enabled, confirmed inputs may be sent to the provider. The deck requires synthetic data and disclosure. I did not open the live Render environment-variable panel, so I cannot say whether a key is currently enabled.

中文理解: PPT 第 20 页说明, 如果启用外部 AI, 已确认的输入可能发送给服务商; 演示要求使用 synthetic data 并进行披露。我没有打开 Render 的环境变量面板, 因此不能判断当前是否启用了 key。

Evidence: PPT slide 20: External AI Data Flow; Render service dashboard checked, environment values not opened.

Limit: Never paste or read the key aloud. Use synthetic values only.

Q: What happens if the AI provider fails?

AI 服务失败时怎么办?

Suggested answer (EN): The current PPT does not describe the provider-failure path. I would not promise a fallback or retry behaviour from the deck alone; I would say this is an implementation detail to confirm with the backend owner.

中文理解: 当前 PPT 没有描述服务商失败时的处理路径。仅凭 PPT 我不会承诺 fallback 或重试行为; 如果被问到, 我会说明这是需要后端负责人确认的实现细节。

Evidence: Not stated in the current PPT; confirm against the live build only if asked.

Limit: Do not present an unverified recovery path as a tested acceptance result.

5. Evidence and honest limitations

These are safe answers when the question goes beyond the visible demo.

Q: Are the current thresholds clinically approved?

当前阈值是否经过临床批准？

Suggested answer (EN): The deck frames approved clinical thresholds as an acceptance requirement, while slide 20 keeps the MVP boundary and non-diagnostic wording visible. It does not show final approval, so I would call the current thresholds illustrative.

中文理解: 演示文稿把 approved clinical thresholds 写成验收要求，同时第 20 页保留 MVP 边界和非诊断措辞。它没有显示最终批准证据，因此我会把当前阈值称为 illustrative。

Evidence: PPT slide 8 acceptance criteria and slide 20 security boundary.

Limit: Say 'illustrative' and 'not a diagnosis' clearly.

Q: Does fasting or random glucose change the result?

空腹或随机血糖会改变结果吗？

Suggested answer (EN): The current PPT does not state how fasting or random glucose changes the result. The deck's MVP flow names manual blood-glucose input, so I would not claim a clinical distinction unless the assessment owner confirms it.

中文理解: 当前 PPT 没有说明 fasting 或 random 会如何改变结果。演示文稿的 MVP 流程只提到手动输入血糖，因此在 assessment owner 确认前我不会声称存在临床区分。

Evidence: Not stated in the current PPT; confirm with the assessment owner.

Limit: Do not imply that the context is already used in a clinical rule.

Q: Do age, sex, smoking and activity affect the current result?

年龄、性别、吸烟和活动量会影响当前结果吗？

Suggested answer (EN): The current PPT does not promise that these fields change the assessment. Slide 10 focuses the MVP flow on blood pressure, cholesterol, glucose and BMI, so I would describe any wider personalisation as open or future scope.

中文理解: 当前 PPT 没有承诺这些字段会改变评估。第 10 页把 MVP 流程集中在血压、胆固醇、血糖和 BMI，因此其他个性化应描述为待确认或未来范围。

Evidence: PPT slide 10: normalised fields BP/cholesterol/glucose/BMI.

Limit: Do not claim personalised risk modelling for these fields yet.

Q: What is still unknown or needs team confirmation?

哪些内容目前未知或需要团队确认？

Suggested answer (EN): I can confirm the current Render service and GitHub main version, and I can see the LeanKit workflow and Huang Guan's cards. I still cannot confirm live database row counts, the current Render AI-key value, final clinical-threshold approval, final AC3.2.5 status or the final QA owner for slide 22 from the evidence available. I would state these as open items rather than guess.

中文理解: 我可以确认当前 Render 服务、GitHub main 版本，以及 LeanKit 的工作流和 Huang Guan 的卡片。但我仍不能从现有证据确认线上数据库行数、当前 Render AI key 值、最终临床阈值批准、AC3.2.5 最终状态，或第 22 页的最终 QA 负责人。我会把它们列为待确认事项，不会猜测。

Evidence: In-app browser checks of Render, GitHub and LeanKit; current 22-slide Google Slides deck.

Limit: This is the correct answer when evidence is missing.

6. Quick evidence card for the presentation

Keep this page open before the session. It maps claims to evidence.

Main version	GitHub main commit 919980d; 0 open pull requests and 12 closed pull requests at the browser check.
Deployed URLs	Frontend: https://healthfirst-team04-frontend.onrender.com Backend API docs: https://healthfirst-team04-api.onrender.com/docs
Render state	Backend and frontend are live from main@919980d; the PostgreSQL service is marked Available. Free services may spin down when idle.
Current PPT	22 slides. Slide 11 shows the database design; slide 20 shows the security boundary; slide 21 shows references; slide 22 is Thank You.
LeanKit state	The board shows Not Started, Started and Finished lanes. Huang Guan's SMART Reflection Card and PM cards are visible; all-criteria completion was not confirmed.
Safety boundary	Use synthetic data only. Do not show the PostgreSQL URL or an OpenRouter key. Do not claim production privacy, clinical approval or PDPA compliance.

Short English phrases to use

"Based on our current evidence, this is a working MVP demonstration, not a production clinical system."

"The result is an illustrative, source-labelled explanation. It is not a diagnosis and the thresholds still need approval."

"We use synthetic values only. The Render dashboard shows the PostgreSQL service as Available, but we do not show credentials or real health data."

"If the evidence is not in the repository, test report or team record, I will call it an open item rather than overclaim it."

Final check before presenting

- Open the latest deck and confirm whether slide 22 changes the final QA ownership.
- Use synthetic values only and do not display the PostgreSQL URL or OpenRouter key.
- If asked about a missing clinical or privacy control, name the limitation and the next review owner.
- Keep the answer short, then offer the evidence file or API contract if the examiner wants details.

Source files checked

Current Google Slides deck (22 slides); Render dashboard for healthfirst-team04-api, healthfirst-team04-frontend and healthfirst-team04-db; GitHub repository main@919980d and pull-request list; LeanKit board 2494590220 and My Cards view; PM_CONTEXT_TEAM04.md and project code/docs used only as background.